PBM Research

What every executive should read before your next contract

EDITION 2026.1 · JULY 2026 · 19 SOURCES

PURPOSE	Primary sources a plan fiduciary can cite, covering PBM economics, pricing, access, and competition
SELECTION	Independent research, government reports, and investigative journalism. Listing a study is not an endorsement of its sponsor
VERIFICATION	Every entry was checked in July 2026: link resolves, the document at that link is the one named, and the description matches what it says
ONLINE	nautilushealth.org/pbm · contractxray.com

The pharmacy benefit industry is not just opaque, it is actively misunderstood. This curated list of independent studies, government reports, and investigative journalism reveals what is really happening behind the scenes: hidden markups, misaligned incentives, anti-competitive contracts, and patient access barriers.

These are not think tank whitepapers gathering dust. These are the studies that change boardroom conversations, drive legislation, and show up in courtrooms.

> **Use them to inform your strategy, educate your team, and raise your expectations.**

GOVERNMENT AND CONGRESSIONAL INVESTIGATIONS

01 **FTC Investigation of Pharmacy Benefit Managers**

FEDERAL TRADE COMMISSION, SECTION 6(B) STUDY, INTERIM STAFF REPORTS JULY 2024 AND JANUARY 2025

The FTC's ongoing study of how the three largest PBMs affect drug pricing, patient access, and independent pharmacies, built on documents obtained under compulsory process. The hub page carries both interim reports and the commissioner statements.

https://www.ftc.gov/terms/pharmacy-benefits-managers-pbm

02 **Specialty Generic Drugs: A Growing Profit Center for Vertically Integrated PBMs**

FEDERAL TRADE COMMISSION, SECOND INTERIM STAFF REPORT, JANUARY 14, 2025

The FTC analyzed 51 specialty generic drugs across 882 National Drug Codes dispensed from 2017 to 2022 and found the three largest PBMs marked up drugs dispensed through their own affiliated pharmacies by hundreds and in some cases thousands of percent. Commercial plan members fared worse than Medicare.

https://www.ftc.gov/reports/specialty-generic-drugs-growing-profit-center-vertically-integrated-pharmacy-benefit-managers

03 The Role of Pharmacy Benefit Managers in Prescription Drug Markets

HOUSE COMMITTEE ON OVERSIGHT AND ACCOUNTABILITY, JULY 23, 2024

A congressional investigation built on more than 140,000 documents obtained from the three largest PBMs, which together control roughly 80 percent of the market. The Committee found the PBMs established group purchasing organizations in Switzerland and Ireland to centralize rebate negotiation, and entities in Ireland and the Cayman Islands to manufacture certain generics and biosimilars.

https://oversight.house.gov/wp-content/uploads/2024/07/PBM-Report-FINAL-with-Redactions.pdf

04 Insulin: Examining the Factors Driving the Rising Cost of a Century Old Drug

US SENATE FINANCE COMMITTEE, GRASSLEY AND WYDEN, JANUARY 14, 2021

A two-year bipartisan investigation reviewing over 100,000 pages of internal documents from three insulin manufacturers and the three largest PBMs. It documented shadow pricing, manufacturers matching each other's increases rather than competing down, and price protection clauses permitting annual increases of up to 12 percent. Manufacturers concluded that lowering list price would reduce what PBMs earned.

https://www.finance.senate.gov/chairmans-news/grassley-wyden-release-insulin-investigation-uncovering-business-practices-between-drug-companies-and-pbms-that-keep-prices-high

05 Report to Congress: Prescription Drug Spending, Pricing Trends, and Premiums

DEPARTMENTS OF LABOR, HEALTH AND HUMAN SERVICES, AND THE TREASURY, USING RXDC DATA

The government report produced from the prescription drug data collection the CAA created, segmented by market. Rebates accounted for 20 to 22 percent of gross drug spending in employer-sponsored and individual market plans, against 31 percent in Medicare Part D and 53 percent in Medicaid. Employers capture the smallest rebate share of any major purchaser.

https://www.dol.gov/sites/dolgov/files/ebsa/laws-and-regulations/laws/no-surprises-act/2024-report-to-congress-prescription-drug-spending.pdf

06 340B Program Hits a Record $81.4 Billion

HRSA AND DRUG CHANNELS INSTITUTE, 2024

HRSA's 2024 data puts 340B discounted purchases at $81.4 billion, up 23 percent in a year and now larger than Medicaid's net drug spend. A channel this large and this opaque belongs on every self-funded plan's radar.

https://www.hrsa.gov/opa/updates/2024-340b-covered-entity-purchases

EMPLOYER ECONOMICS AND CONTRACTING

07 Policy Options To Help Self-Insured Employers Improve PBM Contracting Efficiency

HEALTH AFFAIRS FOREFRONT

Written about self-insured employers specifically, and the clearest published account of why they have not negotiated better terms. An audit of Ohio's Medicaid managed care program found spread pricing accounted for 31 percent of total gross spending on generic drugs.

https://www.healthaffairs.org/content/forefront/policy-options-help-self-insured-employers-improve-pbm-contracting-efficiency

08 2026 Employer Health Care Cost Trend

BUSINESS GROUP ON HEALTH AND SEGAL, 2026 SURVEYS

Large employers project their steepest cost increase in over a decade, with pharmacy running 11 to 12 percent and nearly a quarter of health spend now going to drugs. Both surveys call for transparent, less rebate-dependent PBM models by name.

https://www.businessgrouphealth.org/resources/2026-employer-health-care-strategy-survey-executive-summary

09 **Customer Satisfaction with PBMs at Historic Low**

PSG 2025 PHARMACY BENEFIT MANAGER CUSTOMER SATISFACTION REPORT

Employer and member satisfaction with major PBMs dropped significantly in the 2025 survey, driven by poor service, lack of transparency, and limited member support.

https://www.psgconsults.com/press-release/pharmaceutical-strategies-group-publishes-2025-pharmacy-benefit-manager-customer-satisfaction-report

10 **The Share of Health Care Spending Growth Attributable to Rx**

INITIATIVE ON ENABLING CHOICE AND COMPETITION IN HEALTH CARE, UNIVERSITY OF CHICAGO, SEPTEMBER 2025

Included because it argues against the rest of this list. Prescription drugs accounted for an average of 14.3 percent of annual health spending growth from 2016 to 2024, with hospitals and physician services driving the remainder. Read it before a CFO raises it, because they will.

https://ecchc.economics.uchicago.edu/2025/09/29/policy-brief-the-share-of-health-care-spending-growth-attributable-to-rx/

FIDUCIARY DUTY AND LITIGATION

11 **ERISA Fiduciary Litigation Over Prescription Drug Benefits**

LEWANDOWSKI V. JOHNSON & JOHNSON, NAVARRO V. WELLS FARGO, STERN V. JPMORGAN CHASE

Three class actions alleging plan sponsors failed to monitor their PBMs. Johnson & Johnson was dismissed on Article III standing in January 2025 and again in November 2025. Wells Fargo was dismissed in March 2025 and again in March 2026. JPMorgan partially survived dismissal in March 2026, with the fiduciary breach claims dismissed as plan design but other claims proceeding. Read the holdings rather than the headlines: courts are not rejecting the duty, they are rejecting the injury showing, because participants received the benefits promised.

https://sourceonhealthcare.org/jpmorgan-erisa-drug-cost-suit-partially-survives-motion-to-dismiss/

SPECIALTY DRUGS, GLP-1S, AND BIOSIMILARS

12 **Pharmacy Benefit Alternative Funding Programs: Key Considerations for Self-Funded Plan Sponsors**

MILLIMAN, SHELTON, NIAKAN AND MCBRIDE, APRIL 26, 2024

Alternative funding programs carve high-cost specialty drugs out of coverage and route patients to manufacturer assistance or charity. Milliman documents what that requires of the patient, including income data and credit checks, and finds roughly 32.9 million self-funded lives sit above 500 percent of the federal poverty level and are therefore ineligible for the programs the model depends on. About 14 percent of employers used AFPs in 2023.

https://www.milliman.com/-/media/milliman/pdfs/2024-articles/4-26-24_alternative-funding-whitepaper_final_04262024.ashx

13 **Understanding the GLP-1 Market**

TRUVERIS RESEARCH

For plan sponsors facing the evolving challenge, it is important to understand cost and coverage dynamics across GLP-1 therapies before setting policy.

https://truveris.com/managing-glp-1-coverage/

14 **Real-World GLP-1 Persistence**

PRIME THERAPEUTICS IN JMCP, AND JAMA NETWORK OPEN

Peer-reviewed claims studies find roughly a third of members stay on weight-loss GLP-1s at one year, well below the trial rates plans budget against. The persistence gap is what actually drives the coverage-versus-cost decision.

https://www.jmcp.org/doi/10.18553/jmcp.2026.32.3.281

15 **Utilization Management: Barriers to Care, Not Savings**

AMERICAN MEDICAL ASSOCIATION

Prior authorization and step therapy increasingly delay care, reduce adherence, and often lack clinical justification. The AMA tracks the physician-reported effects and the reform efforts underway.

https://www.ama-assn.org/practice-management/prior-authorization/why-we-fight-fix-prior-authorization

16 **Biosimilar Cost Savings and Formulary Strategy**

AMERICAN JOURNAL OF MANAGED CARE

Plans that adopted biosimilars as preferred agents saved more, saw faster adoption, and avoided formulary disruption.

https://www.ajmc.com/view/strategies-to-boost-biosimilar-adoption-and-improve-patient-access

17 **Why Biosimilar Adoption Lags Despite Cost Advantages**

HEALTH AFFAIRS

Manufacturers and PBMs may discourage biosimilar uptake to preserve rebate flows and exclusive deals, which is why a lower-priced biosimilar can lose formulary position to a higher-priced originator.

https://www.healthaffairs.org/doi/10.1377/hlthaff.2023.01215

18 **Biosimilar Market Trends Report**

CARDINAL HEALTH

Tracks adoption, pricing, and list-to-net spread across biosimilars in both the pharmacy and medical benefit categories.

https://www.cardinalhealth.com/en/product-solutions/pharmaceutical-products/biosimilars/biosimilar-report.html

PRIMARY RESEARCH FROM THIS PROJECT

19 **CAA 2026 Readiness Report**

NAUTILUS HEALTH INSTITUTE, INAUGURAL ISSUE JULY 2026, FOREWORD BY THE NATIONAL ALLIANCE OF HEALTHCARE PURCHASER COALITIONS

Twenty-seven PBM contracts scored against a published standard, provision by provision, with a clause-level citation behind every judgment. Median score 54. Nine contracts reached Excellent, and not one of them started there. Transparent pass-through contracts average 92 while spread-priced contracts average 32.

https://resources.contractxray.com/readiness-report.pdf

Key takeaway

Fiduciary oversight starts with knowing what questions to ask, and what evidence backs them up. These studies connect the dots between PBM profit models and the real-world consequences for employers and participants. Read them. Share them. Cite them.

When someone says that is just how the system works, you will have the receipts to prove it does not have to.

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